BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

Check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle.	Keep an accurate medicine / injection record.	Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.	Continue until the team changes the plan.
Cancer response	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00481 v3: official regimen · Firmagon: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

electrocardiogram (ECG) and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Degarelix: hormone injections every 28 days: treatment and checks

Adult men with advanced hormone-dependent prostate cancer, as covered by NCCP 00481.

The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle.	Keep an accurate medicine / injection record.	Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

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Treatment and review

Two starting injections; then every 28 days

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / act: Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.52